

CONSORT-EHEALTH (V 1.6.1) - Submission/Publication Form

The CONSORT-EHEALTH checklist is intended for authors of randomized trials evaluating web-based and Internet-based applications/interventions, including mobile interventions, electronic games (incl multiplayer games), social media, certain telehealth applications, and other interactive and/or networked electronic applications. Some of the items (e.g. all subitems under item 5 - description of the intervention) may also be applicable for other study designs.

The goal of the CONSORT EHEALTH checklist and guideline is to be

- a) a guide for reporting for authors of RCTs,
- b) to form a basis for appraisal of an ehealth trial (in terms of validity)

CONSORT-EHEALTH items/subitems are MANDATORY reporting items for studies published in the Journal of Medical Internet Research and other journals / scientific societies endorsing the checklist.

Items numbered 1., 2., 3., 4a., 4b etc are original CONSORT or CONSORT-NPT (non-pharmacologic treatment) items.

Items with Roman numerals (i., ii, iii, iv etc.) are CONSORT-EHEALTH extensions/clarifications.

As the CONSORT-EHEALTH checklist is still considered in a formative stage, we would ask that you also RATE ON A SCALE OF 1-5 how important/useful you feel each item is FOR THE PURPOSE OF THE CHECKLIST and reporting guideline (optional).

Mandatory reporting items are marked with a red *.

In the textboxes, either copy & paste the relevant sections from your manuscript into this form - please include any quotes from your manuscript in QUOTATION MARKS, or answer directly by providing additional information not in the manuscript, or elaborating on why the item was not relevant for this study.

YOUR ANSWERS WILL BE PUBLISHED AS A SUPPLEMENTARY FILE TO YOUR PUBLICATION IN JMIR AND ARE CONSIDERED PART OF YOUR PUBLICATION (IF ACCEPTED).

Please fill in these questions diligently. Information will not be copyedited, so please use proper spelling and grammar, use correct capitalization, and avoid abbreviations.

DO NOT FORGET TO SAVE AS PDF _AND_ CLICK THE SUBMIT BUTTON SO YOUR ANSWERS ARE IN OUR DATABASE !!!

Citation Suggestion (if you append the pdf as Appendix we suggest to cite this paper in the caption):

Eysenbach G, CONSORT-EHEALTH Group



CONSORT-EHEALTH: Improving and Standardizing Evaluation Reports of Web-based and Mobile Health Interventions

J Med Internet Res 2011;13(4):e126

URL: <http://www.jmir.org/2011/4/e126/>

doi: 10.2196/jmir.1923

PMID: 22209829

hannahecabre@gmail.com [Switch account](#)

Not shared



Draft saved

*** Indicates required question****Your name ***

First Last

Hannah Cabre

Primary Affiliation (short), City, Country *

University of Toronto, Toronto, Canada

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Title of your manuscript *

Provide the (draft) title of your manuscript.

Examining the Impact of a Co-Developed Multicomponent Mobile e-Health Lifestyle Intervention on Physical Activity and its Association with Gestational Weight Gain in Underserved Women: A State-Wide Randomized Controlled Trial

Name of your App/Software/Intervention *

If there is a short and a long/alternate name, write the short name first and add the long name in brackets.

SmartMoms in WIC

Evaluated Version (if any)

e.g. "V1", "Release 2017-03-01", "Version 2.0.27913"

Your answer

Language(s) *

What language is the intervention/app in? If multiple languages are available, separate by comma (e.g. "English, French")

English

URL of your Intervention Website or App

e.g. a direct link to the mobile app on app in appstore (itunes, Google Play), or URL of the website. If the intervention is a DVD or hardware, you can also link to an Amazon page.

Your answer



URL of an image/screenshot (optional)

Your answer

Accessibility *

Can an enduser access the intervention presently?

- ☐ access is free and open
- ☒ access only for special usergroups, not open
- ☐ access is open to everyone, but requires payment/subscription/in-app purchases
- ☐ app/intervention no longer accessible
- ☐ Other:

Primary Medical Indication/Disease/Condition *

e.g. "Stress", "Diabetes", or define the target group in brackets after the condition, e.g. "Autism (Parents of children with)", "Alzheimers (Informal Caregivers of)"

Gestational Weight Gain

Primary Outcomes measured in trial *

comma-separated list of primary outcomes reported in the trial

Gestational Weight Gain

Secondary/other outcomes

Are there any other outcomes the intervention is expected to affect?

Physical Activity, Sedentary Time

Recommended "Dose" *

What do the instructions for users say on how often the app should be used?

- ☐ Approximately Daily
- ☐ Approximately Weekly
- ☐ Approximately Monthly
- ☐ Approximately Yearly
- ☐ "as needed"
- ☒ Other: There was no app but the intervention was delivered remotely from a i

Approx. Percentage of Users (starters) still using the app as recommended after 3 months *

☒ unknown / not evaluated

☐ 0-10%

☐ 11-20%

☐ 21-30%

☐ 31-40%

☐ 41-50%

☐ 51-60%

☐ 61-70%

☐ 71%-80%

☐ 81-90%

☐ 91-100%

☐ Other:

Overall, was the app/intervention effective? *

☐ yes: all primary outcomes were significantly better in intervention group vs control

☒ partly: SOME primary outcomes were significantly better in intervention group vs control

☐ no statistically significant difference between control and intervention

☐ potentially harmful: control was significantly better than intervention in one or more outcomes

☐ inconclusive: more research is needed

☐ Other:



Article Preparation Status/Stage *

At which stage in your article preparation are you currently (at the time you fill in this form)

- ☐ not submitted yet - in early draft status
- ☐ not submitted yet - in late draft status, just before submission
- ☒ submitted to a journal but not reviewed yet
- ☐ submitted to a journal and after receiving initial reviewer comments
- ☐ submitted to a journal and accepted, but not published yet
- ☐ published
- ☐ Other:

Journal *

If you already know where you will submit this paper (or if it is already submitted), please provide the journal name (if it is not JMIR, provide the journal name under "other")

- ☐ not submitted yet / unclear where I will submit this
- ☒ Journal of Medical Internet Research (JMIR)
- ☐ JMIR mHealth and UHealth
- ☐ JMIR Serious Games
- ☐ JMIR Mental Health
- ☐ JMIR Public Health
- ☐ JMIR Formative Research
- ☐ Other JMIR sister journal
- ☐ Other:

Is this a full powered effectiveness trial or a pilot/feasibility trial? *

☐ Pilot/feasibility

☒ Fully powered

Manuscript tracking number *

If this is a JMIR submission, please provide the manuscript tracking number under "other" (The ms tracking number can be found in the submission acknowledgement email, or when you login as author in JMIR. If the paper is already published in JMIR, then the ms tracking number is the four-digit number at the end of the DOI, to be found at the bottom of each published article in JMIR)

☐ no ms number (yet) / not (yet) submitted to / published in JMIR

☒ Other: 73962

TITLE AND ABSTRACT

1a) TITLE: Identification as a randomized trial in the title

1a) Does your paper address CONSORT item 1a? *

I.e does the title contain the phrase "Randomized Controlled Trial"? (if not, explain the reason under "other")

☒ yes

☐ Other:

1a-i) Identify the mode of delivery in the title

Identify the mode of delivery. Preferably use “web-based” and/or “mobile” and/or “electronic game” in the title. Avoid ambiguous terms like “online”, “virtual”, “interactive”. Use “Internet-based” only if Intervention includes non-web-based Internet components (e.g. email), use “computer-based” or “electronic” only if offline products are used. Use “virtual” only in the context of “virtual reality” (3-D worlds). Use “online” only in the context of “online support groups”. Complement or substitute product names with broader terms for the class of products (such as “mobile” or “smart phone” instead of “iphone”), especially if the application runs on different platforms.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 1a-i? *

Copy and paste relevant sections from manuscript title (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Multicomponent Mobile e-Health Lifestyle Intervention"

1a-ii) Non-web-based components or important co-interventions in title

Mention non-web-based components or important co-interventions in title, if any (e.g., “with telephone support”).

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 1a-ii?

Copy and paste relevant sections from manuscript title (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

The intervention was delivered by mobile phone but there were in person outcome clinic visits

1a-iii) Primary condition or target group in the title

Mention primary condition or target group in the title, if any (e.g., "for children with Type I Diabetes") Example: A Web-based and Mobile Intervention with Telephone Support for Children with Type I Diabetes: Randomized Controlled Trial

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 1a-iii? *

Copy and paste relevant sections from manuscript title (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Underserved Women" is the target group in the title

1b) ABSTRACT: Structured summary of trial design, methods, results, and conclusions

NPT extension: Description of experimental treatment, comparator, care providers, centers, and blinding status.



1b-i) Key features/functionalities/components of the intervention and comparator in the METHODS section of the ABSTRACT

Mention key features/functionalities/components of the intervention and comparator in the abstract. If possible, also mention theories and principles used for designing the site. Keep in mind the needs of systematic reviewers and indexers by including important synonyms. (Note: Only report in the abstract what the main paper is reporting. If this information is missing from the main body of text, consider adding it)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 1b-i? *

Copy and paste relevant sections from the manuscript abstract (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The multicomponent mobile intervention included daily weighing, step tracking with an activity monitor, physical activity and diet counseling, weekly exercise videos with study-provided equipment, health coach interactions, and social support via a private Facebook group."

1b-ii) Level of human involvement in the METHODS section of the ABSTRACT

Clarify the level of human involvement in the abstract, e.g., use phrases like "fully automated" vs. "therapist/nurse/care provider/physician-assisted" (mention number and expertise of providers involved, if any). (Note: Only report in the abstract what the main paper is reporting. If this information is missing from the main body of text, consider adding it)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						



Does your paper address subitem 1b-ii?

Copy and paste relevant sections from the manuscript abstract (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"health coach interactions"

1b-iii) Open vs. closed, web-based (self-assessment) vs. face-to-face assessments in the METHODS section of the ABSTRACT

Mention how participants were recruited (online vs. offline), e.g., from an open access website or from a clinic or a closed online user group (closed usergroup trial), and clarify if this was a purely web-based trial, or there were face-to-face components (as part of the intervention or for assessment). Clearly say if outcomes were self-assessed through questionnaires (as common in web-based trials). Note: In traditional offline trials, an open trial (open-label trial) is a type of clinical trial in which both the researchers and participants know which treatment is being administered. To avoid confusion, use "blinded" or "unblinded" to indicated the level of blinding instead of "open", as "open" in web-based trials usually refers to "open access" (i.e. participants can self-enrol). (Note: Only report in the abstract what the main paper is reporting. If this information is missing from the main body of text, consider adding it)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 1b-iii?

Copy and paste relevant sections from the manuscript abstract (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"351 pregnant women were recruited from the Louisiana Special Supplemental Nutrition program for Women, Infants, and Children (WIC) clinics and were randomly assigned to a multicomponent mobile e-Health intervention for GWG management (N=179) or Usual Care (N=172; standard in-person WIC care) prior to 16 weeks gestation"

1b-iv) RESULTS section in abstract must contain use data

Report number of participants enrolled/assessed in each group, the use/uptake of the intervention (e.g., attrition/adherence metrics, use over time, number of logins etc.), in addition to primary/secondary outcomes. (Note: Only report in the abstract what the main paper is reporting. If this information is missing from the main body of text, consider adding it)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☒	☐	essential
Clear selection						

Does your paper address subitem 1b-iv?

Copy and paste relevant sections from the manuscript abstract (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"351 pregnant women were recruited from the Louisiana Special Supplemental Nutrition program for Women, Infants, and Children (WIC) clinics and were randomly assigned to a multicomponent mobile e-Health intervention for GWG management (N=179) or Usual Care (N=172; standard in-person WIC care) prior to 16 weeks gestation."

1b-v) CONCLUSIONS/DISCUSSION in abstract for negative trials

Conclusions/Discussions in abstract for negative trials: Discuss the primary outcome - if the trial is negative (primary outcome not changed), and the intervention was not used, discuss whether negative results are attributable to lack of uptake and discuss reasons. (Note: Only report in the abstract what the main paper is reporting. If this information is missing from the main body of text, consider adding it)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 1b-v?

Copy and paste relevant sections from the manuscript abstract (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"A decrease in physical activity duration was observed in both groups across pregnancy. The Intervention Group reported an increase in sports participation across pregnancy compared to the Usual Care Group (adjusted effect estimate [95% confidence interval]: +4 MET-hours/week [2; 7], p=0.002). There were no associations between physical activity or sedentary time measures and GWG."

INTRODUCTION

2a) In INTRODUCTION: Scientific background and explanation of rationale



2a-i) Problem and the type of system/solution

Describe the problem and the type of system/solution that is object of the study: intended as stand-alone intervention vs. incorporated in broader health care program? Intended for a particular patient population? Goals of the intervention, e.g., being more cost-effective to other interventions, replace or complement other solutions? (Note: Details about the intervention are provided in "Methods" under 5)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 2a-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Yes, this is addressed in the introduction

2a-ii) Scientific background, rationale: What is known about the (type of) system

Scientific background, rationale: What is known about the (type of) system that is the object of the study (be sure to discuss the use of similar systems for other conditions/diagnoses, if appropriate), motivation for the study, i.e. what are the reasons for and what is the context for this specific study, from which stakeholder viewpoint is the study performed, potential impact of findings [2]. Briefly justify the choice of the comparator.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						



Does your paper address subitem 2a-ii? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Yes, this is addressed in the introduction

2b) In INTRODUCTION: Specific objectives or hypotheses

Does your paper address CONSORT subitem 2b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Given the known health benefits of engaging in regular physical activity and decreasing sedentary time, the objective of this pre-specified secondary analysis was to 1) examine the impact of this pragmatic, multicomponent lifestyle intervention on physical activity and sedentary time, and 2) explore associations between changes in physical activity and sedentary time and GWG."

METHODS

3a) Description of trial design (such as parallel, factorial) including allocation ratio

Does your paper address CONSORT subitem 3a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"This is a secondary analysis of a randomized controlled trial in pregnant women enrolled in Louisiana WIC that was designed to test the effectiveness of a remotely delivered behavior modification program to promote appropriate weight gain during pregnancy as recommended by the 2009 National Academy of Medicine (NAM) GWG per week guidelines (NCT04028843)."

3b) Important changes to methods after trial commencement (such as eligibility criteria), with reasons

Does your paper address CONSORT subitem 3b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The COVID-19 pandemic caused a five-month recruitment pause. To meet the December 2023 enrollment deadline, investigators adjusted the sample size to detect a 16% difference in appropriate weight gain with 80% power, requiring at least 330 participants while maintaining power for secondary outcomes and BMI subgroup analyses."

3b-i) Bug fixes, Downtimes, Content Changes

Bug fixes, Downtimes, Content Changes: ehealth systems are often dynamic systems. A description of changes to methods therefore also includes important changes made on the intervention or comparator during the trial (e.g., major bug fixes or changes in the functionality or content) (5-iii) and other "unexpected events" that may have influenced study design such as staff changes, system failures/downtimes, etc. [2].

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 3b-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the study. We did not use an app.

4a) Eligibility criteria for participants

Does your paper address CONSORT subitem 4a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This information is located under the "participants" section of the manuscript

4a-i) Computer / Internet literacy

Computer / Internet literacy is often an implicit “de facto” eligibility criterion - this should be explicitly clarified.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 4a-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Pregnant women receiving WIC benefits who owned a smartphone with internet access were introduced to the trial in-clinic or via phone or text by the clinic study staff."

4a-ii) Open vs. closed, web-based vs. face-to-face assessments:

Open vs. closed, web-based vs. face-to-face assessments: Mention how participants were recruited (online vs. offline), e.g., from an open access website or from a clinic, and clarify if this was a purely web-based trial, or there were face-to-face components (as part of the intervention or for assessment), i.e., to what degree got the study team to know the participant. In online-only trials, clarify if participants were quasi-anonymous and whether having multiple identities was possible or whether technical or logistical measures (e.g., cookies, email confirmation, phone calls) were used to detect/prevent these.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 4a-ii? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Pregnant women receiving WIC benefits who owned a smartphone with internet access were introduced to the trial in-clinic or via phone or text by the clinic study staff."

4a-iii) Information giving during recruitment

Information given during recruitment. Specify how participants were briefed for recruitment and in the informed consent procedures (e.g., publish the informed consent documentation as appendix, see also item X26), as this information may have an effect on user self-selection, user expectation and may also bias results.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 4a-iii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Interested participants participated in a phone screening call to receive more information about the study and be assessed for eligibility. If participants wished to proceed, an in-person screening visit was scheduled by the clinic staff to review the study consent form and confirm eligibility. Participants were randomized (1:1, parallel-arm) by their assigned health coach after the completion of the first outcome visit to either the Healthy Beginnings mobile e-health intervention in addition to WIC services (Intervention Group) or usual WIC Nutrition only (Usual Care Group), with randomization stratified by body mass index (BMI: normal weight, overweight, obesity) and Louisiana Department of Health (LDH) region."



4b) Settings and locations where the data were collected

Does your paper address CONSORT subitem 4b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"31 WIC clinics across Louisiana"

4b-i) Report if outcomes were (self-)assessed through online questionnaires

Clearly report if outcomes were (self-)assessed through online questionnaires (as common in web-based trials) or otherwise.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 4b-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Participants self-monitored their body weight daily on a cellular network connected scale and their physical activity data with an activity tracker (Fitbit). Participants were contacted weekly by a health coach for one-on-one virtual check-ins through an online messaging platform where participants received personalized feedback on health behaviors related to physical activity and diet. Additionally, social interactions were encouraged in a private Facebook group where tailored videos promoting evidence-based behavioral habits were posted."

"Participant-reported physical activity and sedentary time was collected with the PPAQ."



4b-ii) Report how institutional affiliations are displayed

Report how institutional affiliations are displayed to potential participants [on ehealth media], as affiliations with prestigious hospitals or universities may affect volunteer rates, use, and reactions with regards to an intervention. (Not a required item – describe only if this may bias results)

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☒	☐	essential

Clear selection

Does your paper address subitem 4b-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"All marketing materials demonstrated affiliation with Pennington Biomedical Research Center and WIC."

5) The interventions for each group with sufficient details to allow replication, including how and when they were actually administered

5-i) Mention names, credential, affiliations of the developers, sponsors, and owners

Mention names, credential, affiliations of the developers, sponsors, and owners [6] (if authors/evaluators are owners or developer of the software, this needs to be declared in a "Conflict of interest" section or mentioned elsewhere in the manuscript).

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential

Clear selection



Does your paper address subitem 5-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This is not applicable to the current study.

5-ii) Describe the history/development process

Describe the history/development process of the application and previous formative evaluations (e.g., focus groups, usability testing), as these will have an impact on adoption/use rates and help with interpreting results.

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 5-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This was discussed in the protocol paper and the primary outcome manuscript. "A detailed report of the study design detailing how the study was co-developed with community stakeholders and CONSORT diagram have previously been described."

5-iii) Revisions and updating

Revisions and updating. Clearly mention the date and/or version number of the application/intervention (and comparator, if applicable) evaluated, or describe whether the intervention underwent major changes during the evaluation process, or whether the development and/or content was “frozen” during the trial. Describe dynamic components such as news feeds or changing content which may have an impact on the replicability of the intervention (for unexpected events see item 3b).

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 5-iii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the present trial

5-iv) Quality assurance methods

Provide information on quality assurance methods to ensure accuracy and quality of information provided [1], if applicable.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 5-iv?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Quality assurance methods included a data safety and monitoring plan, double data entry for outcomes, chart auditing, and regular checks for missing data."

5-v) Ensure replicability by publishing the source code, and/or providing screenshots/screen-capture video, and/or providing flowcharts of the algorithms used

Ensure replicability by publishing the source code, and/or providing screenshots/screen-capture video, and/or providing flowcharts of the algorithms used. Replicability (i.e., other researchers should in principle be able to replicate the study) is a hallmark of scientific reporting.

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 5-v?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Detailed information is provided in the published protocol paper in JMIR doi: 10.2196/18211

5-vi) Digital preservation

Digital preservation: Provide the URL of the application, but as the intervention is likely to change or disappear over the course of the years; also make sure the intervention is archived (Internet Archive, [webcitation.org](https://www.webcitation.org), and/or publishing the source code or screenshots/videos alongside the article). As pages behind login screens cannot be archived, consider creating demo pages which are accessible without login.

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 5-vi?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the present trial

5-vii) Access

Access: Describe how participants accessed the application, in what setting/context, if they had to pay (or were paid) or not, whether they had to be a member of specific group. If known, describe how participants obtained "access to the platform and Internet" [1]. To ensure access for editors/reviewers/readers, consider to provide a "backdoor" login account or demo mode for reviewers/readers to explore the application (also important for archiving purposes, see vi).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 5-vii? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"In brief, the Intervention Group received an intensive mobile e-health behavior modification program that targeted healthy GWG. Participants self-monitored their body weight daily on a cellular network connected scale and their physical activity data with an activity tracker (Fitbit). Participants were contacted weekly by a health coach for one-on-one virtual check-ins through an online messaging platform where participants received personalized feedback on health behaviors related to physical activity and diet. Additionally, social interactions were encouraged in a private Facebook group where tailored videos promoting evidence-based behavioral habits were posted. Participants received a weekly 'exercise of the week' clip within the closed Facebook group, and access to eight live exercise sessions that featured pregnant women with likeness to the participant population. Exercise videos featured exercise equipment (yoga mats, dumbbells, resistance bands) that was provided to the participants upon enrollment into the study. Specifically, the physical activity component of the intervention promoted moderate-intensity physical activity of 150 minutes per week, as recommended during pregnancy. The focus of the physical activity component was to reduce sedentary time through increasing lifestyle activities (e.g., reaching 5,000-10,000 steps per day), such as taking the stairs instead of the elevator, actively commuting (e.g., walking or cycling) whenever possible, and replacing sedentary activities with more active options. Health coach feedback was individually tailored to each participants' environment and goals. Physical activity data derived from Fitbit and daily weights derived from the smart scaler were monitored ~weekly by the participants' health coaches."

5-viii) Mode of delivery, features/functionalities/components of the intervention and comparator, and the theoretical framework

Describe mode of delivery, features/functionalities/components of the intervention and comparator, and the theoretical framework [6] used to design them (instructional strategy [1], behaviour change techniques, persuasive features, etc., see e.g., [7, 8] for terminology). This includes an in-depth description of the content (including where it is coming from and who developed it) [1],” whether [and how] it is tailored to individual circumstances and allows users to track their progress and receive feedback” [6]. This also includes a description of communication delivery channels and – if computer-mediated communication is a component – whether communication was synchronous or asynchronous [6]. It also includes information on presentation strategies [1], including page design principles, average amount of text on pages, presence of hyperlinks to other resources, etc. [1].

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 5-viii? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"In brief, the Intervention Group received an intensive mobile e-health behavior modification program that targeted healthy GWG. Participants self-monitored their body weight daily on a cellular network connected scale and their physical activity data with an activity tracker (Fitbit). Participants were contacted weekly by a health coach for one-on-one virtual check-ins through an online messaging platform where participants received personalized feedback on health behaviors related to physical activity and diet. Additionally, social interactions were encouraged in a private Facebook group where tailored videos promoting evidence-based behavioral habits were posted. Participants received a weekly 'exercise of the week' clip within the closed Facebook group, and access to eight live exercise sessions that featured pregnant women with likeness to the participant population. Exercise videos featured exercise equipment (yoga mats, dumbbells, resistance bands) that was provided to the participants upon enrollment into the study. Specifically, the physical activity component of the intervention promoted moderate-intensity physical activity of 150 minutes per week, as recommended during pregnancy. The focus of the physical activity component was to reduce sedentary time through increasing lifestyle activities (e.g., reaching 5,000-10,000 steps per day), such as taking the stairs instead of the elevator, actively commuting (e.g., walking or cycling) whenever possible, and replacing sedentary activities with more active options. Health coach feedback was individually tailored to each participants' environment and goals. Physical activity data derived from Fitbit and daily weights derived from the smart scaler were monitored ~weekly by the participants' health coaches."

5-ix) Describe use parameters

Describe use parameters (e.g., intended "doses" and optimal timing for use). Clarify what instructions or recommendations were given to the user, e.g., regarding timing, frequency, heaviness of use, if any, or was the intervention used ad libitum.

	1	2	3	4	5	
	☐	☐	☐	☐	☒	
subitem not at all important						essential
Clear selection						



Does your paper address subitem 5-ix?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"In brief, the Intervention Group received an intensive mobile e-health behavior modification program that targeted healthy GWG. Participants self-monitored their body weight daily on a cellular network connected scale and their physical activity data with an activity tracker (Fitbit). Participants were contacted weekly by a health coach for one-on-one virtual on an online messaging platform where they received personalized feedback on health behaviors related to physical activity and diet. Additionally, social interactions were encouraged in a private Facebook group where tailored videos promoting evidence-based behavioral habits were posted. Participants received a weekly 'exercise of the week' clip within the closed Facebook group, and access to eight live exercise sessions that featured pregnant women with likeness to the participant population. Exercise videos featured exercise equipment (yoga mats, dumbbells, resistance bands) that was provided to the participants upon enrollment into the study. Specifically, the physical activity component of the intervention promoted moderate-intensity physical activity of 150 minutes per week, as recommended during pregnancy. The focus of the physical activity component was to reduce sedentary time through increasing lifestyle activities (e.g., reaching 5,000-10,000 steps per day), such as taking the stairs instead of the elevator, actively commuting (e.g., walking or cycling) whenever possible, and replacing sedentary activities with more active options. Health coach feedback was individually tailored to each participants' environment and goals. Physical activity data derived from Fitbit were monitored ~weekly by the participants' health coaches."

5-x) Clarify the level of human involvement

Clarify the level of human involvement (care providers or health professionals, also technical assistance) in the e-intervention or as co-intervention (detail number and expertise of professionals involved, if any, as well as "type of assistance offered, the timing and frequency of the support, how it is initiated, and the medium by which the assistance is delivered". It may be necessary to distinguish between the level of human involvement required for the trial, and the level of human involvement required for a routine application outside of a RCT setting (discuss under item 21 – generalizability).

1 2 3 4 5

subitem not at all important ☐ ☐ ☐ ☒ ☐ essential

Clear selection



Does your paper address subitem 5-x?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Both groups received all aspects of the Louisiana WIC program. However, health professionals at WIC did not aid with the mobile e-health intervention offered by the study. The mobile e-health intervention was provided in addition to standard of care from WIC."

5-xi) Report any prompts/reminders used

Report any prompts/reminders used: Clarify if there were prompts (letters, emails, phone calls, SMS) to use the application, what triggered them, frequency etc. It may be necessary to distinguish between the level of prompts/reminders required for the trial, and the level of prompts/reminders for a routine application outside of a RCT setting (discuss under item 21 – generalizability).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 5-xi? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Participants self-monitored their body weight daily on a cellular network connected scale and their physical activity data with an activity tracker (Fitbit). Participants were contacted weekly by a health coach for one-on-one virtual on an online messaging platform where they received personalized feedback on health behaviors related to physical activity and diet. Additionally, social interactions were encouraged in a private Facebook group where tailored videos promoting evidence-based behavioral habits were posted. Participants received a weekly 'exercise of the week' clip within the closed Facebook group, and access to eight live exercise sessions that featured pregnant women with likeness to the participant population."

5-xii) Describe any co-interventions (incl. training/support)

Describe any co-interventions (incl. training/support): Clearly state any interventions that are provided in addition to the targeted eHealth intervention, as ehealth intervention may not be designed as stand-alone intervention. This includes training sessions and support [1]. It may be necessary to distinguish between the level of training required for the trial, and the level of training for a routine application outside of a RCT setting (discuss under item 21 – generalizability).

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 5-xii? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Health coaches had college degrees in nutrition, kinesiology, or psychology and followed a treatment manual under the supervision of trial investigators. Intervention fidelity was maintained through the manual and weekly case conferences.

6a) Completely defined pre-specified primary and secondary outcome measures, including how and when they were assessed

Does your paper address CONSORT subitem 6a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This manuscript is a secondary data analysis. The primary paper reports all CONSORT information <https://doi.org/10.1101/2025.01.29.25321347>

6a-i) Online questionnaires: describe if they were validated for online use and apply CHERRIES items to describe how the questionnaires were designed/deployed

If outcomes were obtained through online questionnaires, describe if they were validated for online use and apply CHERRIES items to describe how the questionnaires were designed/deployed [9].

subitem not at all important 1 2 3 4 5 essential

☐ ☐ ☐ ☐ ☒

Clear selection



Does your paper address subitem 6a-i?

Copy and paste relevant sections from manuscript text

The information is provided under the sub-heading "Movement Context" in the methods section.

6a-ii) Describe whether and how "use" (including intensity of use/dosage) was defined/measured/monitored

Describe whether and how "use" (including intensity of use/dosage) was defined/measured/monitored (logins, logfile analysis, etc.). Use/adoption metrics are important process outcomes that should be reported in any ehealth trial.

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 6a-ii?

Copy and paste relevant sections from manuscript text

"Physical activity data derived from Fitbit and daily weights derived from the smart scaler were monitored ~weekly by the participants' health coaches."

"Participants in the Intervention Group demonstrated engagement with the physical activity component of the intervention. Fitbit data were available from 149 (83%) participants in the Intervention Group. Of the 149 participants, more than half (56%) recorded their steps at least 3 days per week, with over 40% recording their step count on more than 5 days per week on average."



6a-iii) Describe whether, how, and when qualitative feedback from participants was obtained

Describe whether, how, and when qualitative feedback from participants was obtained (e.g., through emails, feedback forms, interviews, focus groups).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 6a-iii?

Copy and paste relevant sections from manuscript text

"Participants were contacted weekly by a health coach for one-on-one virtual on an online messaging platform where they received personalized feedback on health behaviors related to physical activity and diet."

6b) Any changes to trial outcomes after the trial commenced, with reasons

Does your paper address CONSORT subitem 6b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The COVID-19 pandemic caused a five-month recruitment halt. To meet the December 2023 enrollment deadline, the overall sample size was adjusted downward as previously described."

7a) How sample size was determined

NPT: When applicable, details of whether and how the clustering by care provides or centers was addressed



7a-i) Describe whether and how expected attrition was taken into account when calculating the sample size

Describe whether and how expected attrition was taken into account when calculating the sample size.

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 7a-i?

Copy and paste relevant sections from manuscript title (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The COVID-19 pandemic caused a five-month recruitment halt. To meet the December 2023 enrollment deadline, the overall sample size was adjusted downward as previously described."

7b) When applicable, explanation of any interim analyses and stopping guidelines

Does your paper address CONSORT subitem 7b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable since this was a secondary analysis

8a) Method used to generate the random allocation sequence

NPT: When applicable, how care providers were allocated to each trial group

Does your paper address CONSORT subitem 8a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The randomization allocation sequence was concealed to intervention staff prior to group assignment and the trial adhered to a single-blind design with clinic staff and health coaches kept separate."

8b) Type of randomisation; details of any restriction (such as blocking and block size)**Does your paper address CONSORT subitem 8b? ***

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Participants were randomized after the completion of the first outcome visit to either the parallel-arm Healthy Beginnings mobile e-health intervention in addition to WIC services (Intervention Group) or usual WIC Nutrition only (Usual Care Group), with randomization stratified by body mass index (BMI: normal weight, overweight, obesity) and Louisiana Department of Health (LDH) region."

9) Mechanism used to implement the random allocation sequence (such as sequentially numbered containers), describing any steps taken to conceal the sequence until interventions were assigned

Does your paper address CONSORT subitem 9? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The randomization allocation sequence was concealed to intervention staff prior to group assignment and the trial adhered to a single-blind design with clinic staff and health coaches kept separate."

10) Who generated the random allocation sequence, who enrolled participants, and who assigned participants to interventions

Does your paper address CONSORT subitem 10? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Interested participants participated in a phone screening call to receive more information about the study and be assessed for eligibility. If participants wished to proceed, an in-person screening visit was scheduled by the clinic staff to review the study consent form and confirm eligibility. Participants were randomized (1:1, parallel-arm) by their assigned health coach after the completion of the first outcome visit to either the Healthy Beginnings mobile e-health intervention in addition to WIC services (Intervention Group) or usual WIC Nutrition only (Usual Care Group), with randomization stratified by body mass index (BMI: normal weight, overweight, obesity) and Louisiana Department of Health (LDH) region."

11a) If done, who was blinded after assignment to interventions (for example, participants, care providers, those assessing outcomes) and how
NPT: Whether or not administering co-interventions were blinded to group assignment

11a-i) Specify who was blinded, and who wasn't

Specify who was blinded, and who wasn't. Usually, in web-based trials it is not possible to blind the participants [1, 3] (this should be clearly acknowledged), but it may be possible to blind outcome assessors, those doing data analysis or those administering co-interventions (if any).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 11a-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The randomization allocation sequence was concealed to intervention staff prior to group assignment and the trial adhered to a single-blind design with clinic staff and health coaches kept separate."

11a-ii) Discuss e.g., whether participants knew which intervention was the "intervention of interest" and which one was the "comparator"

Informed consent procedures (4a-ii) can create biases and certain expectations - discuss e.g., whether participants knew which intervention was the "intervention of interest" and which one was the "comparator".

	1	2	3	4	5	
subitem not at all important	☐	☐	☒	☐	☐	essential
Clear selection						

Does your paper address subitem 11a-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Participants were informed of the two groups prior to randomization.

11b) If relevant, description of the similarity of interventions

(this item is usually not relevant for ehealth trials as it refers to similarity of a placebo or sham intervention to a active medication/intervention)

Does your paper address CONSORT subitem 11b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This information is provided under the "treatment groups" section of the methods.

12a) Statistical methods used to compare groups for primary and secondary outcomes

NPT: When applicable, details of whether and how the clustering by care providers or centers was addressed

Does your paper address CONSORT subitem 12a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This information is provided under the "statistical analysis" section of the methods.



12a-i) Imputation techniques to deal with attrition / missing values

Imputation techniques to deal with attrition / missing values: Not all participants will use the intervention/comparator as intended and attrition is typically high in ehealth trials. Specify how participants who did not use the application or dropped out from the trial were treated in the statistical analysis (a complete case analysis is strongly discouraged, and simple imputation techniques such as LOCF may also be problematic [4]).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 12a-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Of the 351 enrolled participants, 7% (N = 25) participants were excluded from this analysis due to missing accelerometry data at both time points (i.e., neither early nor late pregnancy accelerometry was available), and an additional 4% (n = 13) were excluded due to an inadequate number (i.e., ≤ 3) of valid days at either time point. Overall, 89% (n = 313) of participants had ≥ 3 valid days for at least one time point, with 84% (n = 294) and 48% (n = 168) having at least three valid days at the early and late pregnancy time points, respectively."

12b) Methods for additional analyses, such as subgroup analyses and adjusted analyses



Does your paper address CONSORT subitem 12b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This information is provided under the "statistical analysis" section of the methods.

X26) REB/IRB Approval and Ethical Considerations [recommended as subheading under "Methods"] (not a CONSORT item)

X26-i) Comment on ethics committee approval

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem X26-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

The study was monitored by the Institutional Review Boards at Pennington Biomedical Research Center and LDH, and participants provided informed consent prior to study procedures.

x26-ii) Outline informed consent procedures

Outline informed consent procedures e.g., if consent was obtained offline or online (how? Checkbox, etc.), and what information was provided (see 4a-ii). See [6] for some items to be included in informed consent documents.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem X26-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Interested participants participated in a phone screening call to receive more information about the study and be assessed for eligibility. If participants wished to proceed, an in-person screening visit was scheduled by the clinic staff to review the study consent form and confirm eligibility."

X26-iii) Safety and security procedures

Safety and security procedures, incl. privacy considerations, and any steps taken to reduce the likelihood or detection of harm (e.g., education and training, availability of a hotline)

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem X26-iii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the present manuscript

RESULTS

13a) For each group, the numbers of participants who were randomly assigned, received intended treatment, and were analysed for the primary outcome
NPT: The number of care providers or centers performing the intervention in each group and the number of patients treated by each care provider in each center

Does your paper address CONSORT subitem 13a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Information can be found in Table 1.

13b) For each group, losses and exclusions after randomisation, together with reasons

Does your paper address CONSORT subitem 13b? (NOTE: Preferably, this is shown in a CONSORT flow diagram) *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Of the 351 enrolled participants, 7% (N = 25) participants were excluded from this analysis due to missing accelerometry data at both time points (i.e., neither early nor late pregnancy accelerometry was available), and an additional 4% (n = 13) were excluded due to an inadequate number (i.e., ≤ 3) of valid days at either time point. Overall, 89% (n = 313) of participants had ≥ 3 valid days for at least one time point, with 84% (n = 294) and 48% (n = 168) having at least three valid days at the early and late pregnancy time points, respectively."

13b-i) Attrition diagram

Strongly recommended: An attrition diagram (e.g., proportion of participants still logging in or using the intervention/comparator in each group plotted over time, similar to a survival curve) or other figures or tables demonstrating usage/dose/engagement.

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 13b-i?

Copy and paste relevant sections from the manuscript or cite the figure number if applicable (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Attrition can be found in the primary paper.

14a) Dates defining the periods of recruitment and follow-up

Does your paper address CONSORT subitem 14a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Briefly, recruitment took place from July 2019 to December 2023 at 31 WIC clinics across Louisiana, selected by size and location."

14a-i) Indicate if critical "secular events" fell into the study period

Indicate if critical "secular events" fell into the study period, e.g., significant changes in Internet resources available or "changes in computer hardware or Internet delivery resources"

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 14a-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The COVID-19 pandemic caused a five-month recruitment halt. To meet the December 2023 enrollment deadline, the overall sample size was adjusted downward as previously described."

14b) Why the trial ended or was stopped (early)



Does your paper address CONSORT subitem 14b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the present trial.

15) A table showing baseline demographic and clinical characteristics for each group

NPT: When applicable, a description of care providers (case volume, qualification, expertise, etc.) and centers (volume) in each group

Does your paper address CONSORT subitem 15? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Information can be found in Table 1

15-i) Report demographics associated with digital divide issues

In ehealth trials it is particularly important to report demographics associated with digital divide issues, such as age, education, gender, social-economic status, computer/Internet/ehealth literacy of the participants, if known.

	1	2	3	4	5	
	☐	☐	☐	☐	☒	
subitem not at all important						essential
Clear selection						

Does your paper address subitem 15-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

This information was provided in Table 1 for the current secondary analysis

16) For each group, number of participants (denominator) included in each analysis and whether the analysis was by original assigned groups

16-i) Report multiple "denominators" and provide definitions

Report multiple "denominators" and provide definitions: Report N's (and effect sizes) "across a range of study participation [and use] thresholds" [1], e.g., N exposed, N consented, N used more than x times, N used more than y weeks, N participants "used" the intervention/comparator at specific pre-defined time points of interest (in absolute and relative numbers per group). Always clearly define "use" of the intervention.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 16-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Of the 351 enrolled participants, 7% (N = 25) participants were excluded from this analysis due to missing accelerometry data at both time points (i.e., neither early nor late pregnancy accelerometry was available), and an additional 4% (n = 13) were excluded due to an inadequate number (i.e., ≤ 3) of valid days at either time point. Overall, 89% (n = 313) of participants had ≥ 3 valid days for at least one time point, with 84% (n = 294) and 48% (n = 168) having at least three valid days at the early and late pregnancy time points, respectively."

16-ii) Primary analysis should be intent-to-treat

Primary analysis should be intent-to-treat, secondary analyses could include comparing only "users", with the appropriate caveats that this is no longer a randomized sample (see 18-i).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 16-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Physical activity data were analyzed using a linear mixed model including fixed effects for time (early and late pregnancy) and group (Intervention Group, Usual Care) as well as the interaction, and a random effect for participant. The model was adjusted for BMI category (normal weight, overweight, obesity) at randomization. Model-derived estimated marginal means alongside corresponding 95% confidence intervals as well as within- and between-group effect estimates with associated confidence intervals and p-values are reported. The main effect of interest is the between-group effect estimate. Furthermore, the treatment heterogeneity across the different BMI categories was explored via a three-way interaction including time, group, and BMI category, with custom comparisons exploring intervention effects on physical activity within and between BMI categories (Supplemental Table 1)."

17a) For each primary and secondary outcome, results for each group, and the estimated effect size and its precision (such as 95% confidence interval)

Does your paper address CONSORT subitem 17a? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Model-derived estimated marginal means alongside corresponding 95% confidence intervals as well as within- and between-group effect estimates with associated confidence intervals and p-values are reported."

17a-i) Presentation of process outcomes such as metrics of use and intensity of use

In addition to primary/secondary (clinical) outcomes, the presentation of process outcomes such as metrics of use and intensity of use (dose, exposure) and their operational definitions is critical. This does not only refer to metrics of attrition (13-b) (often a binary variable), but also to more continuous exposure metrics such as "average session length". These must be accompanied by a technical description how a metric like a "session" is defined (e.g., timeout after idle time) [1] (report under item 6a).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 17a-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Participants in the Intervention Group demonstrated engagement with the physical activity component of the intervention. Fitbit data were available from 149 (83%) participants in the Intervention Group. Of the 149 participants, more than half (56%) recorded their steps at least 3 days per week, with over 40% recording their step count on more than 5 days per week on average. On average, participants achieved their step goal of 5,000 and 10,000 steps per day on $39 \pm 26\%$ and $8 \pm 13\%$, respectively, of the days that they recorded steps. Participants took an average of 4560 ± 2168 steps per day across the intervention period, which was consistent between the second (4785 ± 2293) and third (4602 ± 2231) trimesters."

17b) For binary outcomes, presentation of both absolute and relative effect sizes is recommended

Does your paper address CONSORT subitem 17b? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

The present analysis had no statistical testing on binary outcomes.

18) Results of any other analyses performed, including subgroup analyses and adjusted analyses, distinguishing pre-specified from exploratory

Does your paper address CONSORT subitem 18? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"To examine the effect of changes in physical activity on GWG outcomes (study-observed GWG, weekly GWG, and GWG guideline attainment), we used data from all participants collapsed across intervention groups (i.e., independent of randomization assignment). These analyses used a linear model including a fixed effect for the change across the intervention period in both MVPA and inactivity (adjusting for each other in the same model) as measured via Actigraph and adjustments for BMI category at randomization, parity (categorically defined as nulliparous vs non-nulliparous), and gestational age at enrolment. Effects are presented as regression coefficients (study-observed GWG, weekly GWG) or odds ratios (GWG guideline attainment) for each predictor with 95% confidence intervals and corresponding p-values."

18-i) Subgroup analysis of comparing only users

A subgroup analysis of comparing only users is not uncommon in ehealth trials, but if done, it must be stressed that this is a self-selected sample and no longer an unbiased sample from a randomized trial (see 16-iii).

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential

Clear selection

Does your paper address subitem 18-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Not applicable to the present manuscript.

19) All important harms or unintended effects in each group
 (for specific guidance see CONSORT for harms)
Does your paper address CONSORT subitem 19? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

There were no harms or unintended effects in each group

19-i) Include privacy breaches, technical problems

Include privacy breaches, technical problems. This does not only include physical “harm” to participants, but also incidents such as perceived or real privacy breaches [1], technical problems, and other unexpected/unintended incidents. “Unintended effects” also includes unintended positive effects [2].

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 19-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks “like this” to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Your answer

19-ii) Include qualitative feedback from participants or observations from staff/researchers

Include qualitative feedback from participants or observations from staff/researchers, if available, on strengths and shortcomings of the application, especially if they point to unintended/unexpected effects or uses. This includes (if available) reasons for why people did or did not use the application as intended by the developers.

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 19-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Your answer

DISCUSSION

22) Interpretation consistent with results, balancing benefits and harms, and considering other relevant evidence

NPT: In addition, take into account the choice of the comparator, lack of or partial blinding, and unequal expertise of care providers or centers in each group

22-i) Restate study questions and summarize the answers suggested by the data, starting with primary outcomes and process outcomes (use)

Restate study questions and summarize the answers suggested by the data, starting with primary outcomes and process outcomes (use).

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential

Clear selection

Does your paper address subitem 22-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Results are presented for each outcome variable as "baseline," "change within groups," and Change between groups."

22-ii) Highlight unanswered new questions, suggest future research

Highlight unanswered new questions, suggest future research.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 22-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Our intervention provided items to assist utilization of their existing physical environment (e.g., equipment, home-based exercise videos) and social environment (e.g., Facebook group). However, larger policy, systems, and large-scale environmental factors may be needed to maintain or increase physical activity in economically disadvantaged populations."

20) Trial limitations, addressing sources of potential bias, imprecision, and, if relevant, multiplicity of analyses

20-i) Typical limitations in ehealth trials

Typical limitations in ehealth trials: Participants in ehealth trials are rarely blinded. Ehealth trials often look at a multiplicity of outcomes, increasing risk for a Type I error. Discuss biases due to non-use of the intervention/usability issues, biases through informed consent procedures, unexpected events.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 20-i? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Yet, our study has limitations related to study conduct, movement behavior assessment, and varying interaction levels with the intervention. First, our study was conducted during the COVID-19 pandemic, which may have negatively influenced physical activity habits in our population. Second, the self-report physical activity method may be biased based on participant perception of their daily activities and their health literacy. However, the use of the Actigraph as a device measure of physical activity strengthens the current methodology, and using self-report and device-based measures is considered the best practice for holistic evaluation of physical activity. Even so, there is no consensus on processing data from wrist-worn devices, definition of valid days, or use of cut-points/threshold considerations, though we used methods and analysis software aligning with previous research."

21) Generalisability (external validity, applicability) of the trial findings

NPT: External validity of the trial findings according to the intervention, comparators, patients, and care providers or centers involved in the trial



21-i) Generalizability to other populations

Generalizability to other populations: In particular, discuss generalizability to a general Internet population, outside of a RCT setting, and general patient population, including applicability of the study results for other organizations

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem 21-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"Possible reasons we did not see an effect of change in MVPA or movement behaviors on GWG in the present study may be due to greater physical demands from home chores, family responsibilities, and physically intensive jobs, rather than structured exercise, in this population. Additionally, other lifestyle interventions in socioeconomically disadvantaged individuals have cited barriers to success such as unsafe neighborhoods limiting the ability to exercise, time demands of occupation, and lack of access to exercise equipment. Our intervention provided items to assist utilization of their existing physical environment (e.g., equipment, home-based exercise videos) and social environment (e.g., Facebook group). However, larger policy, systems, and large-scale environmental factors may be needed to maintain or increase physical activity in economically disadvantaged populations."

21-ii) Discuss if there were elements in the RCT that would be different in a routine application setting

Discuss if there were elements in the RCT that would be different in a routine application setting (e.g., prompts/reminders, more human involvement, training sessions or other co-interventions) and what impact the omission of these elements could have on use, adoption, or outcomes if the intervention is applied outside of a RCT setting.

	1	2	3	4	5	
subitem not at all important	☒	☐	☐	☐	☐	essential
Clear selection						

Does your paper address subitem 21-ii?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Your answer

OTHER INFORMATION

23) Registration number and name of trial registry

Does your paper address CONSORT subitem 23? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Clinical Trial Registration: NCT04028843



24) Where the full trial protocol can be accessed, if available

Does your paper address CONSORT subitem 24? *

Cite a Multimedia Appendix, other reference, or copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

Full trial protocol is available in JMIR Research Protocols doi: 10.2196/18211

25) Sources of funding and other support (such as supply of drugs), role of funders

Does your paper address CONSORT subitem 25? *

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The authors would like to thank the Louisiana Department of Health, Louisiana Women's, Infants and Children's program staff for their support and the participants for their time and effort. This research was supported by funding from the National Institute of Nursing Research (5R01NR017644), the Louisiana/Pennington Nutrition and Obesity Research Center (NORC; P30DK072476), the Louisiana Clinical and Translational Sciences Center (U54 GM104940) and postdoctoral fellowship grants to EWF (F32HD108022), CLK (K99HD107158, P20GM144269), HEC (T32DK064584), and EKW (F32HL176217)."

X27) Conflicts of Interest (not a CONSORT item)

X27-i) State the relation of the study team towards the system being evaluated

In addition to the usual declaration of interests (financial or otherwise), also state the relation of the study team towards the system being evaluated, i.e., state if the authors/evaluators are distinct from or identical with the developers/sponsors of the intervention.

	1	2	3	4	5	
subitem not at all important	☐	☐	☐	☐	☒	essential
Clear selection						

Does your paper address subitem X27-i?

Copy and paste relevant sections from the manuscript (include quotes in quotation marks "like this" to indicate direct quotes from your manuscript), or elaborate on this item by providing additional information not in the ms, or briefly explain why the item is not applicable/relevant for your study

"The authors declare no conflicts of interest."

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As a result of using this checklist, did you make changes in your manuscript? *

- ☐ yes, major changes
- ☒ yes, minor changes
- ☐ no

What were the most important changes you made as a result of using this checklist?

Added recruitment information

How much time did you spend on going through the checklist INCLUDING making ^{*} changes in your manuscript

It took approximately 3 hours

As a result of using this checklist, do you think your manuscript has improved? ^{*}

- ☒ yes
- ☐ no
- ☐ Other:

Would you like to become involved in the CONSORT EHEALTH group?

This would involve for example becoming involved in participating in a workshop and writing an "Explanation and Elaboration" document

- ☐ yes
- ☒ no
- ☐ Other:

Clear selection



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